

Name : Mr. JALUM MATTO
Lab No. : 463577618
Ref By : DR ADEEB SINGH
Collected : 19/12/2024 2:05:00PM
A/c Status : P
Collected at : Hexamed Healthcare Services LLP
25-270/6, Taluka Bahu, Main Bypass Road, NH1A,
Channi Himmat,
Jammu,

Age : 38 Years
Gender : Male
Reported : 21/12/2024 1:23:48PM
Report Status : Final
Processed at

Test Report

STONE ANALYSIS BY FTIR



Sample: Kidney Stones

Aggregate Weight: 0.032 gm

Composition : Calcium Oxalate Monohydrate 90% Calcium Oxalate Dihydrate 10%

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Jammu Address: 25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

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Test Report (Representative stone analysed)

Kidney stone (Nephrolithiasis) develops when urine becomes supersaturated with crystal forming substances such as Calcium, uric acid, oxalate etc. (resulting from dehydration or genetic predisposition) or due to decrease in urinary concentration of substances such as magnesium, citrate etc. that inhibit stone formation. Kidney stones tend to recur; approximately 50% of patients who form one stone form another in 10 years. Nephrolithiasis is considered to be a disease of affluence because it is more prevalent in conditions like Obesity, Hypertension & Diabetes. For patients with multiple kidney stones 24-hour urine chemistries should be measured to identify abnormalities that predispose to kidney stones, which guides dietary and pharmacologic therapy to prevent future stone events.

STONE TYPE	FREQUENCY (%)	CAUSES
Calcium oxalate / mix	75	- Hypercalciuria with Hypercalcemia (Hyperparathyroidism, Vitamin D excess, malignancy) - Idiopathic hypercalciuria - Hypocitraturia - Alkaline urine, low urine volume - High dose of vitamin C - Metabolic syndrome - Inflammatory bowel disease
Calcium Phosphate	5	- Distal Renal Tubular acidosis - Primary Hyperparathyroidism - Chronic urinary tract infection - Hypercalciuria and/or Hypophosphaturia
Magnesium Ammonium Phosphate (Struvite)	10-20	Recurrent urinary tract infection
Uric Acid	5-10	- Hyperuricosuria and /hyperuricosemia - Metabolic syndrome (Diabetes, obesity, Hypertension) - Acidic urine (pH<5.5) - Gout
Cystine	1	Cystinuria (A genetic disorder)

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DIETARY RECOMMENDATIONS FOR PREVENTION OF RECURRENCE

General Dietary measures:

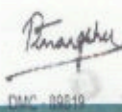
- Fluid intake should be between 2.5-3L/day or it should be sufficient to produce urine output of 2L/day
- Diet rich in fibres, fruits (orange, lemon, apple etc) & vegetables
- Maintain adequate calcium intake (1000-1200 mg/day)
- Aim to achieve/maintain healthy weight

Specific measures based on metabolic evaluation and stone composition:

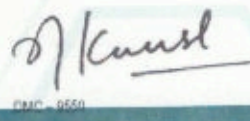
STONE COMPONENT	METABOLIC PROFILE	SUGGESTED DIETARY RECOMMENDATION
Calcium oxalate or combined calcium oxalate/phosphate stone	Hypercalciuria	Salt restriction
	Hyperoxaluria	Calcium supplements with meal Restrict items rich in oxalate eg. Soya protein, Spinach, Nuts, Strawberries, Cocoa, Chocolates,
Uric Acid	Hyperuricosuria	beetroot, carrot, radish etc • Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom • Reduce alcohol intake
	Hypocitraturia	Increase citrate rich food (Orange, lemon, apple etc)
	Hyperuricosuria & Hyperuricemia	• Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom • Reduce alcohol intake
Cystine	Metabolic syndrome	Lifestyle changes, weight loss, Diabetes control
		• Increase fluid intake to 3-4L, Urine output should at least be 3L • Restrict sodium & protein intake


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